Foreword

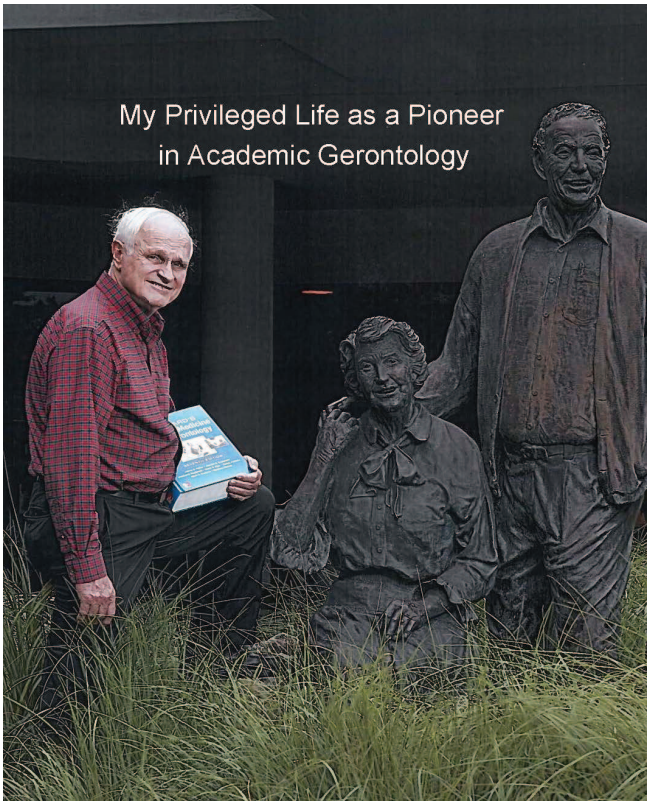
"My inspiration and my passion will always come from my older patients. One day I hope to be like them; when that day comes, I hope that my doctor will be a geriatrician."[1]

It has been my privilege as Editor Emeritus of this latest edition to follow its progress over the past 2 years from design at the first meeting of the editorial board in September 2019 to publication. I am pleased to predict that the eighth edition of *Hazzard's Geriatric Medicine and Gerontology* will solidify its reputation as the leading "go to" source for readers seeking the latest, most comprehensive, in-depth, and reliable treatise in research and practice that optimizes the care of our aging and older patients. Moreover, this edition will continue to expand its influence toward worldwide enhancement in the care of those patients, whose numbers are expanding dramatically as population aging becomes a global phenomenon.

Especially gratifying in my association with this textbook has been its widespread use as core material in geriatric fellowship training programs. I am frequently flattered to be asked to autograph the latest volume for fellows who have come to value the book highly during their training for its accessibility, breadth, depth, and authenticity as a single source for their scholarly education in our field.

Their appreciation brings me special satisfaction. A focused approach on geriatrics fellows in training as our primary target audience was adopted by Reubin Andres, Ed Bierman, and me when, in 1983, we were charged by McGraw Hill to collaborate as editors of the first edition of a new textbook in the field, *Principles of Geriatric Medicine*. We envisioned our textbook as complementing the prior education that physicians received in medical school, graduate training in residency, and then maintained in continuing education programs, education that forms the foundation of physician care for all patients. In retrospect, that thoughtful, deliberate approach to defining our project generated the most important and enduring attribute of our series as it grew and matured through its successive editions: specifically, that our textbook should focus on the needs of future leaders in this new field, those who would choose to undergo training at the post-graduate level—that is, fellows in Gerontology and Geriatric Medicine.

Through our textbook we have hoped that physicians will be enriched and continuously kept current from its concentration upon the special needs of their older patients. Clinical topics have been integrated with chapters summarizing the aging process across the lifespan from conception to death (gerontology), but honing down on those older than 65, 75 (my personal definition as the threshold to old age), and, perhaps most germane to our special contributions as geriatricians, the "oldest old," those older than 85 (the group to which I now belong!) in whom the art and science of caring for the most complex and vulnerable can be most appreciated. And with the "Aging Tsunami" of retiring baby-boomers upon us, our mission to enhance the practice of all professionals and caregivers in managing such patients through our textbook is all the more urgent. Thus, it has been gratifying to observe the



[1]Hazzard WR. I am a geriatrician. *JAGS*. 2004; 52:161.

expansion of our original focus on physicians training in geriatrics to encompass the educational needs of trainees in the wide range of health professions that are required to meet the clinical challenges presented by older adults with complex health care problems.

I am especially pleased that one of the editors of the eighth edition, Stephanie Studenski, has worked with other leaders in the field to document the range of geriatrics training experiences for physicians and other health professionals as summarized in the new chapter, Geriatrics Around the World. This special expertise will serve them well as future leaders in health care as it becomes progressively skewed toward caring for older patients. Thus, it will be incumbent upon editors of future editions of this textbook to remain abreast of leading edge developments in our field, which promises to become ever more sophisticated and challenging as aging citizens continue to enjoy increasing healthspan as well as lifespan.

This textbook remains the most enduring icon of my career in the field of Gerontology and Geriatric Medicine. It is with pride and confidence that I predict that the education of future generations of all clinicians and researchers who focus on the care of older patients will be enhanced by the eighth edition and future editions of this textbook.

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